Chapter 2: Understanding Human Behaviour and Social Determinants in Social and Behaviour Change Communication

1.0 Introduction.

The discipline of Social and Behaviour Change Communication (SBCC) represents a profound paradigm shift within the broader fields of public health, international development, and environmental conservation. Historically, communication strategies deployed to address complex societal challenges relied heavily on top-down, unidirectional approaches.

These earlier frameworks were frequently conceptualised under the umbrella of Information, Education, and Communication (IEC) or traditional Behaviour Change Communication (BCC). Such legacy models operated on a linear assumption: that individuals engaged in harmful practices simply because they lacked correct information, and therefore, disseminating factual knowledge would naturally and inevitably precipitate a change in behaviour.

Contemporary academic research and decades of programmatic evidence, however, demonstrate unequivocally that human behaviour is rarely a simple, rational product of information acquisition. Instead, human action is deeply embedded within a complex, dynamic web of individual psychologies, interpersonal relationships, entrenched cultural norms, and overriding structural environments.

Consequently, SBCC has evolved beyond mere message dissemination. It is formally defined in academic literature as a research-based, consultative process that strategically employs tailored communication approaches to promote and facilitate changes in knowledge, attitudes, norms, beliefs, and behaviours.

Crucially, unlike its predecessors, SBCC explicitly encompasses social change perspectives. It recognizes that to alter individual behaviour sustainably, practitioners must simultaneously support the requisite social and structural changes necessary to create an enabling environment. This involves fostering processes of community dialogue, advocacy, and collective action to shift the underlying determinants of health and well-being.

SBCC operates synergistically across multiple levels of society. It integrates sophisticated tactics derived from commercial marketing, social mobilisation, entertainment-education (edutainment), policy advocacy, and interpersonal communication.

To deploy SBCC effectively, practitioners must possess a highly nuanced understanding of the determinants that drive, facilitate, or inhibit human action. This chapter provides a comprehensive exploration of these determinants for aspiring SBCC practitioners. It outlines the foundational theories of behavioural psychology and sociology, dissects the specific cognitive and social variables that constitute human decision-making.

2.0 Individual, Social and Environmental Factors Influencing Behaviour

Human behaviour cannot be understood, analysed, or altered in a vacuum. It is the cumulative result of dynamic, continuous interactions between factors operating at multiple interconnected strata. Sustainable behaviour change generally requires interventions that accurately identify and systematically address barriers across three primary domains: the individual, the social,

and the environmental. To master SBCC, students must first understand the distinct mechanisms operating within each of these domains.

At the individual level, behaviour is influenced by internal characteristics and intricate cognitive processes. These include an individual's inherent genetics, emotional states, factual knowledge, evaluative attitudes, deep-seated beliefs, and practical skills. A critical component functioning at this level is self-efficacy, which is defined as an individual's intrinsic belief in their own perceived ability to perform a specific action or behaviour successfully.

Self-efficacy dictates the amount of effort a person will expend toward a goal and their persistence in the face of inevitable obstacles. Furthermore, personal risk perceptions, such as an individual's subjective belief regarding their susceptibility to a specific health threat and the severity of that threat, are powerful cognitive drivers of individual action. However, human agency is bounded. Even with phenomenally high self-efficacy, comprehensive knowledge, and a strong desire to change, an individual may fail to adopt a desired behaviour if external factors present insurmountable barriers.

The social level encompasses the interpersonal interactions, relationships, and networks that surround and encapsulate an individual. This includes family members, peers, spouses, partners, and the broader, immediate community. Human beings are inherently social creatures, evolved to seek group cohesion.

Consequently, individual decisions are heavily mediated by the expectations, emotional support, and judgements of their immediate social circles. Social factors include peer influence, community leadership dynamics, social capital, and collective efficacy.

Collective efficacy refers to the shared belief within a community in its conjoint capabilities to organise and execute the courses of action required to produce given levels of attainments. When a specific behaviour is highly stigmatised within a community, an individual is highly unlikely to adopt it, regardless of their personal cognitive beliefs regarding its benefits, out of a fear of social exclusion.

The environmental level, frequently referred to in sociological literature as the structural, systemic, or macro-level, relates to the broader context in which individuals and communities exist. This domain encompasses the physical environment, such as geography, climate, and built infrastructure, as well as the sociopolitical environment, which includes government policies, macroeconomic conditions, health system architectures, resource availability, media landscapes, and institutional protocols.

Environmental factors can act as profound deterrents or powerful enablers of behaviour. For example, an individual may possess the knowledge and desire to use modern contraception (individual level), and their social network may support this choice (social level); however, if the local health clinic is geographically inaccessible, financially prohibitive, or frequently experiences stock-outs of contraceptive supplies (environmental level), the behaviour simply cannot be enacted.

To illustrate the complex, interdependent interplay of these factors, students must examine rigorous academic evaluations of public health interventions. A definitive case study demonstrating these dynamics is the evaluation of a school-engaged SBCC approach for malaria prevention in Ethiopia, conducted by researchers at Jimma University between 2017

and 2019 and documented in the BMC Public Health journal. Malaria remains a severe, life-threatening public health challenge in Ethiopia, with over 70% of the country classified as a risk area and 52% of the population at direct risk, necessitating robust, decentralised collective action. The researchers systematically evaluated the individual, social, and environmental factors influencing the engagement, empowerment, and retention of the education sector (primarily teachers and frontline health providers) in local malaria elimination efforts.

At the individual level, the researchers measured the cognitive and psychological variables of the participating stakeholders. Through rigorous statistical modelling, they found that self-efficacy was a robust, dominant predictor of the SBCC programme's acceptability ($\beta = 0.438$) and feasibility ($\beta = 0.352$). Stakeholders who held strong, internalised beliefs in their personal capability to perform the required malaria prevention activities were significantly more likely to support and implement the intervention. Additionally, the perceived threat of malaria, an individual cognitive assessment of environmental danger, strongly motivated participation ($\beta = 0.40$ for acceptability).

At the social level, the SBCC intervention actively leveraged interpersonal communication between students, teachers, and families to influence broader community norms. The academic study revealed that perceived community support was the absolute strongest predictor of the programme's feasibility ($\beta = 0.591$). The degree to which stakeholders felt the community valued, respected, and supported their efforts directly dictated the perceived viability of the SBCC strategy, demonstrating that individual intent is highly reliant on social validation.

Crucially, the Jimma University study also highlighted highly unexpected, counterintuitive dynamics at the environmental and contextual level. The researchers assessed the 'school climate' as a localized environmental factor. Paradoxically, the data revealed that a highly positive existing school climate was negatively associated with the acceptability ($\beta = -0.16$) and feasibility ($\beta = -0.099$) of the new SBCC programme.

The researchers posited that in schools where the environment was already highly structured, successful, and positive, staff might be more sceptical of new, external mandates. This resistance may stem from fears of overburdening staff, historical experiences with high staff turnover, or philosophical misperceptions regarding whether educational institutions should be responsible for frontline healthcare provision.

Furthermore, the broader structural environment, including the 'three-tier' health care system of Ethiopia and the critically low rates of mass media exposure (with 68% of women and 53% of men having zero exposure to mass media), underscored the absolute necessity of utilizing localized physical infrastructure, like schools, as interpersonal hubs for SBCC, as national media campaigns would structurally fail to reach the target audience. This case study demonstrates that individual intent is insufficient without social backing, and that structural environments can yield complex influences on programme reception.

2.1 The Social-Ecological Model and its Application in SBCC

To systematically map, diagnose, and address the myriad, overlapping factors influencing human behaviour, SBCC scholars and practitioners rely heavily on established theoretical frameworks. The most prominent and foundational of these is the Social-Ecological Model (SEM). Informed heavily by the seminal work of developmental psychologist Urie Bronfenbrenner in 1979, the SEM posits that behaviour is a complex phenomenon shaped by

the continuous, reciprocal interaction between individuals and their multifaceted environments. Social ecology, in this academic context, is defined as the study of the influence of the social context on behaviour, explicitly including institutional, structural, and cultural variables.

The SEM provides an indispensable diagnostic framework that prevents communication practitioners from myopically focusing solely on individual-level messaging. It fundamentally challenges the assumption that behaviour is an isolated, autonomous, rational choice, emphasising instead that individuals are deeply embedded within concentric rings of influence. While specific adaptations of the SEM vary slightly depending on the academic discipline (e.g., psychology vs. public health), in SBCC it is generally operationalised across four to five interconnected levels.

To aid in conceptualisation, the following table delineates the standard levels of the SEM and their direct corresponding applications within SBCC strategy design:

SEM Level	**Definition and Scope**	**SBCC Application and Strategic Focus**
Individual (Intrapersonal)	The internal characteristics of the individual that influence behaviour change. This encompasses genetics, factual knowledge, attitudes, beliefs, self-efficacy, emotional intelligence, and developmental history.	Tailored interpersonal counselling, targeted health education materials, and digital health applications aimed specifically at increasing individual knowledge and shifting personal attitudes.
Interpersonal (Network)	Formal and informal social networks and social support systems. This includes family members, peers, romantic partners, close friends, and immediate social circles that provide identity and support.	Peer education programmes, family and couples counselling, and communication initiatives specifically targeting spousal communication, joint decision-making, and household power dynamics.
Community	The situational settings in which social relationships occur, including neighbourhoods, schools, workplaces, and religious institutions. This level encompasses social capital, local leadership, and community norms.	Community mobilisation events, participatory town hall dialogues, and intensive engagement with traditional and religious leaders to shift local norms and foster collective efficacy.
Organisational / Institutional	The formal and informal rules, regulations, policies, and structures within organisations that constrain or promote recommended behaviours among their members or constituents.	Internal advocacy and capacity-building training within institutions (e.g., healthcare clinics, universities, schools) to improve service delivery, quality of care, and institutional protocols.
Public Policy / Societal	Broad structural, political, macro-economic, and cultural determinants. Includes local, state, and national laws, economic policies, national media environments, gender income equity, and deeply ingrained cultural ideologies.	Mass media campaigns, national policy advocacy, large-scale entertainment-education (edutainment), and strategic efforts to shift macroeconomic or legal frameworks.

The core tenet of the SEM in SBCC asserts that interventions are exponentially more effective when they deploy a multi-pronged, integrated strategy that synchronises communication tactics across several levels simultaneously. A communication campaign attempting to change individual behaviour without addressing the restrictive social norms at the community level, or the structural barriers at the public policy level, is highly likely to fail.

A definitive, peer-reviewed academic case study demonstrating the robust, multi-level application of the SEM in SBCC is the Nigerian Urban Reproductive Health Initiative (NURHI), a large-scale project funded by the Bill & Melinda Gates Foundation. Nigeria historically faced profound structural, cultural, and social barriers to modern contraceptive use, resulting in persistently high maternal mortality rates and unintended pregnancies. To combat this, NURHI was meticulously designed utilising the Theory of Ideation, which posits that behaviour is strongly influenced by cognitive, emotional, and social interactions, and mapped its interventions directly onto the SEM to systematically shift deeply entrenched social norms.

At the individual level, NURHI aimed to alter personal cognitive attitudes towards family planning (FP) and dispel pervasive, localized myths regarding the physiological side effects of contraceptives. Moving outward to the interpersonal level, the programme explicitly addressed marital relationship dynamics, specifically targeting the social norm regarding a woman's agency and ability to use contraception in the face of spousal disagreement.

At the community and societal levels, NURHI deployed intensive, well-funded demand-generation activities to alter perceived social norms, specifically targeting what an individual believes other people will think of them if they are known to use family planning. To achieve this sweeping multi-level influence, NURHI utilised a diverse media mix: mass media (television and radio spots for societal reach), entertainment-education (scripted radio and television dramas to model positive behaviours and interpersonal negotiations regarding FP), and direct community social mobilisation.

By consistently engaging the individual psyche, the household dynamic, and the broader societal discourse simultaneously, researchers found that the resulting shifts in social norms regarding the spacing of pregnancies and delaying first pregnancies were significantly and positively related to the increased voluntary use of modern contraceptives among women in the target cities.

Another highly illustrative application of the SEM is found in academic analyses of lifestyle behaviours, such as physical activity among undergraduate university students. While physical activity is universally recognised as vital for holistic well-being, university students consistently report alarmingly low participation rates, which detrimentally impacts their mental health and academic motivation.

A critical narrative review applying the SEM to this demographic demonstrated that individual psychosocial factors (such as mental health status and intrinsic motivation) were heavily mediated by higher-level variables. At the interpersonal level, a lack of active social support from peers served as a significant deterrent. At the organisational and environmental levels, barriers such as severe academic pressures, inadequate campus fitness resources, and unsupportive built environments structurally constrained the students' ability to exercise, regardless of their individual motivation or knowledge regarding the benefits of exercise.

The review concluded that university policies must move beyond simply telling students to exercise; they must adopt a multi-level SEM approach, combining individual psychosocial support with the physical restructuring of campus environments and the fostering of peer support networks, to effectively promote physical activity.

Furthermore, the SEM facilitates cross-sectoral integration in SBCC. In Guatemala, the Health Communication Capacity Collaborative (HC3) developed an integrated SEM for the Western Highlands Integrated Program (WHIP). By applying the SEM, HC3 unified projects across diverse sectors, including health, nutrition, hygiene, agriculture, food production, and water use, recognising that the structural determinants (like water access and agricultural yield) directly impacted individual health behaviours (like hygiene and nutrition). This holistic, multi-level approach strengthened the effectiveness of the programmes by fostering greater citizen participation among the indigenous populations and promoting deeply sustainable development.

3.0 Concepts of Knowledge, Attitudes, Beliefs, Norms, and Practices in SBCC

At the very core of individual and social transformation in SBCC is the intricate interplay between Knowledge, Attitudes, Beliefs, Norms, and Practices (often conceptualised in research and evaluation frameworks as KAP or KABNP). For decades, rudimentary communication strategies conflated these concepts, treating them as interchangeable variables and assuming that an increase in one automatically equated to an increase in the others.

In SBCC scholarship, however, each term holds a distinct operational definition, is measured differently, and occupies a specific, sequential place on the continuum of behaviour change.

To fully grasp the mechanics of SBCC, students must internalise the distinctions between these variables:

Concept	**Academic Definition within SBCC**	**Programmatic Implication**
Knowledge	Factual, objective information that an individual possesses regarding a specific topic, derived from education or credible information sources.	Providing accurate knowledge is foundational for informed choice but is rarely sufficient on its own to drive complex behaviour change.
Beliefs	What a person subjectively holds to be true, regardless of empirical evidence, objective fact, or scientific consensus. Often rooted in culture or religion.	Inaccurate beliefs (myths and misconceptions) act as severe barriers. SBCC must carefully unpick these without causing cognitive dissonance or offending cultural sensibilities.
Attitudes	An individual's evaluative disposition—positive, negative, or neutral—toward a specific object, person, issue, or proposed behaviour.	Attitudes are influenced by both knowledge and beliefs. Fostering a positive attitude is a crucial intermediate step before a behaviour is attempted.
Norms	The unwritten, implicit social and gender rules that dictate what is considered acceptable, expected, or appropriate behaviour within a given society or peer group.	Individuals often act in accordance with norms to maintain social standing, even if their personal attitudes conflict with the norm. Normative change requires community-level dialogue.
Practices	The actual, observable, routinized actions undertaken by individuals. Distinguished from a singular	The ultimate goal of SBCC. Measuring practices requires observational data or

'behaviour', a practice implies habituation and social sharing.	longitudinal self-reporting, moving beyond simply asking what people know.

Knowledge, in the context of health communication, is considered the bedrock of informed choice, ensuring that individuals understand their options, the mechanisms of action, and potential side effects. For example, in family planning, possessing accurate knowledge about human fertility cycles and the specific physiological side effects of various contraceptive methods is critical. Individuals equipped with this comprehensive knowledge are significantly more likely to initiate and sustain family planning practices compared to those who possess only superficial awareness of a method's mere existence.

Beliefs, conversely, represent deeply ingrained cognitive constructs that often emerge from cultural socialisation, religious teachings, or the assimilation of inaccurate knowledge over generations.

In public health contexts, inaccurate beliefs frequently manifest as persistent myths and misconceptions. For instance, an individual might possess the factual knowledge that a contraceptive implant prevents pregnancy, but simultaneously hold the unscientific, culturally transmitted belief that the implant causes permanent sterility, encourages promiscuity, or results in a dangerous buildup of 'bad blood' due to amenorrhea (menstrual cycle obstruction).

In SBCC, addressing beliefs is vastly different from addressing knowledge; simply providing a pamphlet with more medical facts is rarely sufficient to dislodge a deeply entrenched cultural belief.

Attitudes are directly influenced by the interaction of knowledge and beliefs. If an individual holds the inaccurate belief that a specific health practice causes permanent harm to their fertility, they will naturally and logically develop a highly negative attitude toward that practice, thereby creating a formidable psychological barrier to its adoption, regardless of what medical professionals advise.

Norms, specifically social and gender norms, exert an incredibly powerful gravitational pull on human behaviour. People are heavily influenced by, and deeply concerned about, the opinions and actions of those around them. Even the most robust, scientifically accurate, and well-funded individually targeted communication campaign cannot overpower deeply ingrained social norms.

When an individual's newly formed positive attitude conflicts with prevailing, restrictive social norms, the social norm typically prevails due to the fundamental human desire for social integration and the potent fear of isolation and social sanction.

A poignant and distressing case study illustrating the profound, overriding influence of norms over knowledge and attitudes revolves around the academic research on harmful practices, specifically child marriage and Female Genital Mutilation (FGM) in countries such as Uganda, Ethiopia, and Kenya.

FGM has zero medical health benefits and causes severe, lifelong physical and psychological trauma; however, in certain tribal communities, it is a deeply entrenched practice performed as a mandatory rite of passage for girls entering puberty, tied inextricably to purity and marriageability.

The academic research highlights a highly complex and contradictory normative landscape. Through quantitative surveys, researchers discovered that caregivers and parents often possess the cognitive knowledge that education is highly valuable for a girl's future economic prospects. However, deeply rooted cultural beliefs regarding sexual purity and marital readiness foster a negative attitude toward delaying marriage. The prevailing social and gender norms dictate that post-puberty, women are expected to abandon schooling and prioritize domestic responsibilities and child-rearing, regardless of the age of marriage.

Most concerningly, the Ugandan case study reveals the dynamic, shifting nature of norms. FGM is not merely a dying legacy practice in isolated regions; it is actively being adopted by entirely new ethnicities and generations that have not historically practiced it. This adoption is driven by shifting inter-community social norms and the desire for social cohesion and marriageability among neighbouring tribes.

Furthermore, research in Kenya highlights the role of structural and community leadership in perpetuating these norms. While religious leaders and village elders are cited as the most important source of information on FGM and child marriage (influencing the norm), these leaders often lack basic knowledge regarding sexual and reproductive health rights and the severe medical repercussions of the practices. Addressing such deeply entrenched practices requires SBCC interventions that move far beyond individual knowledge dissemination, which has historically been criticized by audiences as "preachy," "scolding," and "finger-wagging".

Instead, interventions must facilitate intensive, long-term community-wide dialogues that systematically redefine the cultural norms of womanhood, passage, and readiness for marriage without alienating the community.

4.0 Descriptive vs. Injunctive Norms

Within SBCC literature, a critical theoretical distinction must be made between two distinct types of social norms: descriptive norms and injunctive norms. Failing to distinguish between these two variants during the design phase of a campaign can lead to catastrophic communication failures and unintended negative consequences.

Descriptive norms refer strictly to perceptions of what other people actually do in reality. They are based on the observation of typical behaviour in a given context (e.g., observing that "Most people in this community litter on the sidewalk"). Injunctive norms, on the other hand, refer to perceptions of what behaviours are socially approved or disapproved of by the group. They represent the moral and ethical rules of the society (e.g., understanding that "People in this community strongly disapprove of littering, even if it happens").

When designing SBCC campaigns to combat a widespread issue, practitioners often instinctively attempt to raise public awareness by highlighting the sheer prevalence and gravity of the problem. Unfortunately, this strategy inadvertently amplifies the descriptive norm, communicating to the audience that the negative behaviour is highly common, thereby unintentionally normalizing it.

An academic analysis of major anti-corruption messaging campaigns in developing countries, such as Kenya, Nigeria, and Mexico, vividly illustrates this psychological phenomenon. International donors and civil society groups frequently fund massive billboard campaigns warning of the systemic, endemic nature of bribery and corruption, fulfilling mandates to "raise

public awareness" regarding the threat. While the explicit intent is to highlight the gravity of the issue (aligning with the injunctive norm that corruption is morally bad), the messaging inadvertently emphasizes the descriptive norm that absolutely everyone is engaging in corruption. Decades of research demonstrate that calling attention to descriptive norms causes people to behave more in accordance with that norm. Consequently, making the public acutely aware that corruption is ubiquitous actually makes individuals less willing to fight it and statistically more prone to engage in bribery themselves, as they perceive corruption as the inescapable, standard operating procedure of their society.

A remarkably similar effect was documented in a classic, widely cited psychological case study regarding environmental theft at Arizona's Petrified Forest National Park. Park authorities, alarmed by the depletion of resources, placed prominent signage warning new arrivals that: "Your heritage is being vandalized every day by theft losses of petrified wood of 14 tons a year, mostly a small piece at a time". By attempting to induce guilt and highlight the sheer severity of the problem, the signage effectively communicated a highly powerful descriptive norm: stealing wood is a common, widespread, and typical activity for visitors. The communication strategy inadvertently legitimized the theft by demonstrating that "everyone else is doing it."

The firm academic recommendation for such situations, particularly those characterized by high levels of socially censured or illegal conduct, is to strictly avoid focusing the audience's attention on the descriptive norm. Instead, SBCC practitioners must carefully craft messages that align exclusively with the injunctive norm, highlighting the severe social disapproval and the ethical imperatives of the approved behaviour, thereby utilizing positive social pressure and moral framing to drive change.

5.0 Introduction to Behavioural Psychology and Sociology Relevant to SBCC

To systematically alter human practices, SBCC does not rely on intuition; it draws heavily upon rigorous, empirically tested theories and frameworks derived from both behavioural psychology and sociology. While psychology predominantly focuses on the internal cognitive processes, mental schemas, and the individual locus of control, sociology examines the external structural determinants, routine social interactions, and power dynamics that shape human existence. A sophisticated SBCC strategy almost universally integrates theories from both disciplines to address both the mind of the individual and the structural realities of their environment.

5.1 Psychological Theories in SBCC

Behavioural psychology offers a multitude of models that map the complex cognitive pathways leading to human action. Two of the most widely applied and empirically validated theories in SBCC are the Theory of Planned Behaviour (TPB) and Social Cognitive Theory (SCT).

The Theory of Planned Behaviour, developed by psychologist Icek Ajzen, posits that human action is guided by three core, interacting considerations:

1. **Behavioural beliefs**, which produce a favourable or unfavourable attitude toward the specific behaviour.
2. **Normative beliefs**, which result in perceived social pressure or subjective norms regarding the behaviour.

3. **Control beliefs**, which give rise to perceived behavioural control, a concept highly akin to self-efficacy, representing the individual's belief in their capability to execute the behaviour despite barriers.

According to the TPB, these three factors coalesce to form a strong 'behavioural intention'. The theory dictates that the stronger the intention to engage in a behaviour, the more likely should be its actual performance. A contemporary academic study examining public compliance with protective behaviours during the severe public health emergency of the COVID-19 pandemic applied the TPB to predict behaviour among UK adults. The researchers found that individuals' intentions, which were heavily influenced by their confidence that they could actually manage to engage in the behaviour (perceived behavioural control) and their instrumental attitudes toward the efficacy of the measures, directly and accurately predicted later, real-world compliance with maintaining physical distance and limiting social visits.

The TPB is also routinely used to test mobile applications; for example, utilizing theory-based 'behavior change techniques' (BCTs) to motivate users to switch to pro-environmental, energy-saving heating behaviours in their homes.

Social Cognitive Theory, pioneered by Albert Bandura, diverges from purely internal, cognitive models by emphasizing the dynamic, continuous interaction between personal factors, environmental influences, and human behaviour, a concept Bandura formally termed 'reciprocal determinism'. A highly unique construct of SCT is 'observational learning' (or modelling), which suggests that individuals acquire new behaviours not just through direct experience, but by observing others (role models) and witnessing the consequences of their actions. Furthermore, SCT heavily centralises self-efficacy as the primary, indispensable engine of behaviour change.

A comprehensive scoping review of health promotion interventions deployed in primary care settings demonstrated the profound, widespread efficacy of SCT in clinical and community environments. Reviewing 39 peer-reviewed journal articles, researchers found that interventions explicitly leveraging SCT constructs, specifically enhancing 'self-efficacy' through goal setting and utilizing 'observational learning' through trained peer role models, resulted in significant, measurable positive health outcomes.

These outcomes included increased adherence to complex post-transplant medication regimens, successful transitions to healthy dietary lifestyles, and marked reductions in high-risk behaviours related to STI transmission.

Similarly, the *StrongPeople – Healthy Weight* initiative, a community-based intervention developed at Tufts University, utilized SCT principles to reduce cardiovascular disease risk among overweight and obese women over the age of 40. By integrating skill-building exercises to boost self-efficacy and utilizing group settings for peer observation, the programme achieved significant decreases in participant body mass index and substantial increases in sustained physical activity.

Furthermore, community care organizations utilize SCT in small-group interventions for people living with HIV/AIDS, using skill-building exercises to increase independence and develop long-term healthy habits.

5.2 Sociological Theories in SBCC

While psychological theories excel at explaining individual cognitive intent, sociology asserts that human behaviour is inextricably linked to overarching social structures, institutional power, and historical contexts. Sociological frameworks argue that focusing solely on individual cognition often results in 'victim-blaming', punishing individuals for failing to adopt a behaviour when structural, economic, or cultural barriers render the behaviour impossible.

A prominent sociological framework utilized in the design of SBCC is Practice Theory, particularly the complex concept of *Habitus* developed by French sociologist Pierre Bourdieu. Habitus refers to a collection of deeply ingrained tendencies, cognitive biases, values, and even bodily dispositions held by an individual, which are learned slowly through mimesis and intense socialisation from peers of a similar socioeconomic background. Habitus operates largely below the level of conscious, rational intention; it provides the "feel for the game" that makes certain cultural behaviours feel entirely natural and alternative behaviours virtually unthinkable.

Everyday social interactions continuously develop and enforce these social practices, which in turn shape the macro social structure. Closely related is Social Capital Theory, also defined by Bourdieu (1985), which emphasizes the aggregate of actual or potential resources that are linked to the possession of a durable network of institutionalized relationships of mutual acquaintance or recognition. In the context of SBCC, leveraging social capital means systematically utilizing trusted community networks, local leaders, and established groups to diffuse innovations and shift normative practices from within, rather than imposing them from the outside.

A masterful, large-scale application of sociological principles in SBCC is found in India's POSHAN Abhiyaan (Prime Minister's Overarching Scheme for Holistic Nutrition). Launched to address severe, intractable national challenges of stunting, undernutrition, anaemia, and low birth weight among children and pregnant women, the Indian government recognized that dietary practices are not merely a matter of biological knowledge. Instead, they are deeply informed by geography, rigid social norms, cultural beliefs, and structural economic conditions. Rather than merely distributing nutritional pamphlets to individual mothers (a purely psychological, knowledge-deficit approach), the massive programme was explicitly engineered as a *Jan Andolan*—a mass, transformative social movement.

The *Jan Andolan* strategy utilized sociological mechanisms to promote change. It aimed to alter the collective habitus surrounding dietary behaviours by improving the structural linkages between grassroots communities and formal health systems. The integrated SBCC campaigns validated and heavily promoted culturally proximal "promising practices" that aligned seamlessly with local traditions, utilizing social capital to shift the normative expectations surrounding the critical first 1,000 days of life (from a woman's pregnancy until her child's second birthday). The strategic focus was deliberately shifted from the individual mother as the sole locus of change, to the community as the locus of change, thereby transforming the distribution of responsibility, support, and power within the social fabric to ensure nutritional security.

Similarly, the Malawi BRIDGE Project for HIV/AIDS prevention provides a compelling, evidence-based synthesis of individual and sociological SBCC strategies. Operating between

2003 and 2009, BRIDGE moved far beyond the basic, traditional BCC messaging of condom promotion. Recognizing that HIV transmission was heavily dictated by social factors, the project addressed the underlying structural determinants of vulnerability, including severe gender inequality, entrenched social stigma, and restrictive community norms regarding sexual dialogue. By evaluating the project through rigorous, matched household surveys utilizing non-intervention groups for comparison, academic researchers demonstrated that fostering open community dialogue and addressing the macro sociological drivers of the epidemic, rather than isolating the individual patient, yielded highly measurable improvements in HIV prevention, stigma reduction, and treatment adherence in resource-limited settings.

Furthermore, edutainment programmes like the *Soul City* television series in South Africa demonstrate the power of sociological media interventions. By depicting realistic community responses to gender-based violence (GBV) on a mass scale, the series bypassed individual lecturing and instead modelled new community norms, significantly increasing public attention to and collective action against GBV by utilizing the mass media to shift the societal discourse.

6.0 Conclusion

The historical transition from basic, unidirectional information dissemination to the sophisticated, multidisciplinary approach of Social and Behaviour Change Communication is fundamentally predicated on academically rigorous understanding of human behaviour. As demonstrated throughout this chapter, the decision to adopt, maintain, or reject a specific behaviour is a complicated calculations. It is driven intrinsically by individual psychologies, emotional states, and cognitive beliefs; it is constrained or enabled by immediate social networks and cultural norms; and it is bounded by broad, inescapable environmental, economic, and structural realities.

By systematically applying frameworks such as the Social-Ecological Model, future SBCC practitioners can accurately diagnose these diverse determinants and design holistic interventions that resonate simultaneously across the individual, interpersonal, and societal strata. Understanding the nuanced, critical distinctions between factual knowledge and deeply held beliefs, or the dramatically divergent psychological impacts of descriptive versus injunctive norms, empowers communicators to avoid the common catastrophic pitfalls of counterproductive messaging.

Furthermore, by seamlessly integrating the internal cognitive insights of behavioural psychology with the external, structural, and normative focus of sociology, SBCC transcends mere advertising or public relations. It becomes a vital catalyst for profound, sustainable, and equitable transformation, uniquely capable of addressing the most pressing health and developmental challenges of the 21st century by reshaping not just what individuals know, but the very social realities in which they live and operate.

Question for discussion / debate.

Debate Topic 1: The Locus of Change – Individual Agency vs. Structural Determinants

Question: "This house believes that SBCC programmes should prioritise funding for structural and environmental changes over interventions aimed at individual cognitive and psychological factors."

Debate Topic 2: The Ethics of Messaging – Descriptive vs. Injunctive Norms

Question: "In campaigns addressing highly prevalent, harmful societal issues, it is more effective to strictly promote the socially desired behaviour (injunctive norms) rather than raising public awareness about the severe reality and scale of the problem (descriptive norms)."

Debate Topic 3: Cultural Relativity vs. Universal Health Rights

Question: "When deeply entrenched cultural norms and beliefs contradict established human rights and public health standards, external SBCC practitioners are justified in actively intervening to dismantle these local traditions."